FIELD PLAYBOOK

Modern Pharmaceutical Sales Strategies

Reverse negotiation · financial modelling · behavioural psychology

Six strategies · six institutional deliverables · one Product Battlecard



AGENDA

PSYCHOLOGY FINANCE SIMULATION

Six strategies, one shift: from pitching to proving

1

FBI Negotiation

Lead the client to a “safe no”, then let calibrated questions do the work.

PSYCHOLOGY

2

Pharmonomics

Talk shelf return and cash flow with the pharmacist, not discounts.

FINANCE

3

Behavioural Nudge Matrix

Arrange the choice so the right option feels natural and low-risk.

PSYCHOLOGY

4

Business War Games

Rehearse sudden market shocks in real time, under pressure.

SIMULATION

5

Red Teaming

Attack your own product before a competitor does.

SIMULATION

6

AI Persona Simulator

Practise on a virtual physician and get scored on five dimensions.

SIMULATION

CAPSTONE

The Product Battlecard

One page that brings all six strategies together, updated every quarter

OVERVIEW

Traditional marketing, rebuilt as an advanced discipline

BUILT ON THREE PILLARS

- **Reverse negotiation**
- **Financial modelling**
- **Behavioural psychology**

FIVE MOVES THAT REPLACE THE TRADITIONAL PITCH

1. **Tactical empathy & accusation audits**
   disarm healthcare providers before the pitch begins
2. **Portable Pharmonomics calculators**
   show pharmacists real inventory profitability and cash-flow gains
3. **AI persona simulators**
   replicate diverse physician personalities for training and performance evaluation
4. **Red teaming & business war games**
   prepare teams to counter aggressive competition and sudden industry disruption
5. **Behavioural nudge matrices & micro-expression analysis**
   help reps navigate cognitive biases and guide decisions without direct friction

STRATEGY 1

FBI Negotiation Methodology

Tactical empathy · accusation audit · calibrated open questions · the Negotiation Playbook



STRATEGY 1 · CONCEPT

From persuasion to reverse negotiation and risk management

TRADITIONAL PERSUASION

The rep pushes the pitch and tries to force the physician or pharmacist to accept it. Closed questions invite a defensive “yes” or “no”.

REVERSE NEGOTIATION · TACTICAL EMPATHY

The rep leads the client to a **“safe no”** that gives them a feeling of control. Calibrated questions then guide them to solve the rep’s challenge themselves.

STEP 1

Accusation audit

Voice their objections first

STEP 2

A “safe no”

The client feels in control

STEP 3

Calibrated questions

Open “how” questions invite collaboration

OUTCOME

They solve it

The client resolves the rep’s challenge themselves

NOTE A “safe no” is not a lost visit. Giving the client control lowers their defences, which is what makes the calibrated questions that follow work.

STRATEGY 1 · TECHNIQUE

The accusation audit: say their objection before they do

LECTURE EXAMPLE · OPENING LINE

“Doctor, you probably think I’m coming to repeat the same daily scientific pitch you hear all the time, and that our product is higher priced without justification.”

BATTLECARD VERSION · NAMES THE COMPETITOR

“Doctor / Pharmacist, before we begin, you probably think I’m here to repeat the same standard sales pitch you hear every day, and that our product is priced higher without offering a true clinical advantage over `[Competitor Brand]`*.”*

HOW IT WORKS

1. Anticipate the worst thoughts the client already holds
2. Say them first, at the opening of the visit

✓ **Impact:** the client’s defensive posture is neutralised

NOTE Replace `[Competitor Brand]` before use, and keep the tone sincere: the audit only works if it sounds honest, not sarcastic.

STRATEGY 1 · TECHNIQUE

Calibrated open questions: let the client solve it

REMOVED

~~Closed questions that end in “yes” or “no”~~

REPLACED WITH

Reverse, open-ended **“how”** questions that invite the doctor or pharmacist to collaborate on the solution, and guide them to resolve the objection themselves.

NOTE Each question starts with “how” and points at the client’s problem, never at the product.

GENERAL

“**How** can I support you in cases that do not respond to the first line of treatment?”

GENERAL

“**How** can we overcome the price barrier with the patient without affecting treatment quality?”

PHYSICIANS · BATTLECARD

“**How** do you currently manage patient cases that fail to achieve target clinical endpoints on first-line therapy?”

PHARMACISTS · BATTLECARD

“**How** can we structure an order that maximises your shelf turnover rate without tying up your monthly cash flow?”

STRATEGY 1 · DELIVERABLE

The FBI Negotiation Playbook

#	OBJECTION OR ADMISSION	PAIRED CALIBRATED QUESTION
01	**“You’ll repeat the same daily scientific pitch.”**	Open with the accusation audit before presenting.
02	**“Your product is priced higher without justification.”**	How can we overcome the price barrier with the patient without affecting treatment quality?
03	**Patients not responding to first-line treatment**	How can I support you in cases that do not respond to the first line of treatment?
04	**Pharmacy cash flow tied up in stock**	How can we structure an order that maximises your shelf turnover rate without tying up your monthly cash flow?
05	Objection to be mapped from field visits	Calibrated question to be written
06	Objection to be mapped from field visits	Calibrated question to be written
07	Objection to be mapped from field visits	Calibrated question to be written
08	Objection to be mapped from field visits	Calibrated question to be written
09	Objection to be mapped from field visits	Calibrated question to be written
10	Objection to be mapped from field visits	Calibrated question to be written

TO DO The playbook maps the top 10 most complex market objections. Rows 1–4 come from this material; rows 5–10 must be filled in by the team.

STRATEGY 2

Pharmonomics

GMROI vs simple margin · opportunity cost · the 3-input calculator · free cash flow proof



STRATEGY 2 · CONCEPT

Shelf return, not simple margin

GROSS MARGIN RETURN ON INVENTORY

$$GMROI = \frac{\text{Gross margin}}{\text{Average inventory cost}}$$

Profit earned for every unit of money tied up on the shelf.

Slow mover 30% margin · 2 turns a year `0.86`

Fast mover 20% margin · 6 turns a year `1.50`



Illustrative figures · GMROI = margin × turns at cost ÷ (1 − margin)

Move the visit away from price

Pharmacy visits shift from basic discount discussions to **data-driven financial modelling and shelf investment analysis**. Product turnover drives the pharmacy's bottom line and cash flow.

€ **Opportunity cost**

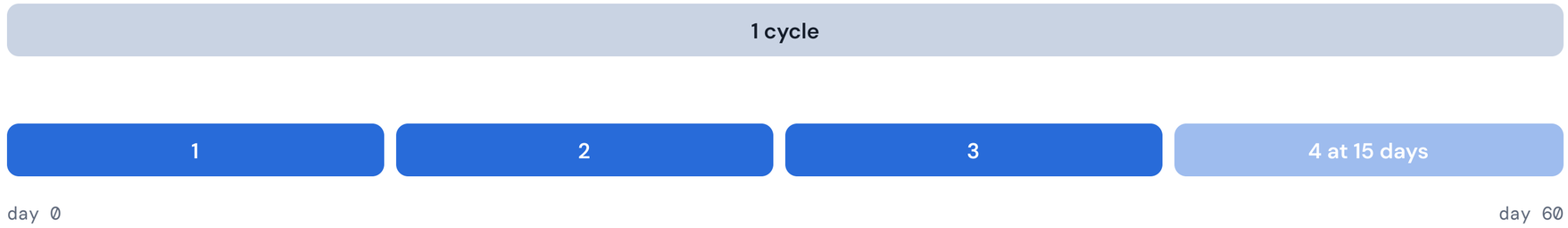
Every slow-turning competitor product on the shelf is money the pharmacy could be earning elsewhere. The rep makes that cost visible.

NOTE A lower margin can still win. The example shows a 20% product returning more per unit of stock than a 30% one, because it turns 3× faster.

STRATEGY 2 · EVIDENCE

Turnover speed: the number that frees working capital

ONE 60-DAY WINDOW ON THE SHELF



GROSS MARGIN %

Competitor: **high** (deep discount). Ours: **standard**.

The competitor looks cheaper at first.

TURNOVER RATE

Competitor: **60 days** (slow). Ours: **15–20 days** (fast).

Ours frees up working capital 3–4× faster.

GMROI · CASH FLOW

Competitor: lower overall yield. Ours: **higher free cash flow**.

Reallocating 0.5 m of shelf raises monthly cash flow.

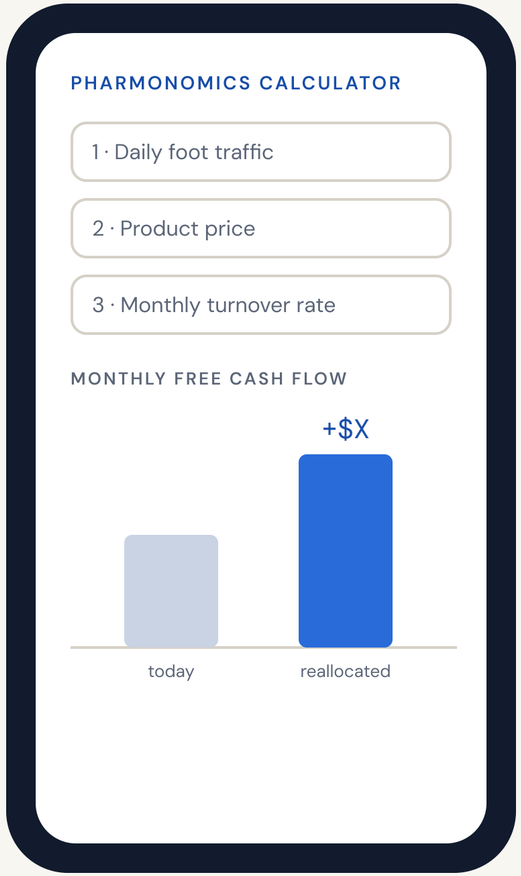
NOTE This comparison is qualitative. Before quoting numbers to a pharmacist, replace it with that pharmacy's own figures from the calculator.

STRATEGY 2 · TOOL

The Pharmonomics calculator: three inputs, one chart

EXCEL SHEET OR MOBILE APP, USED IN FRONT OF THE PHARMACIST

1. **Daily pharmacy foot traffic**
2. **Product price**
3. **Expected monthly turnover rate**



THE FREE CASH FLOW PROOF

"If you reallocate half a metre of shelf space to our product instead of a slower-moving competitor item, your monthly free cash flow will increase by $X."

NOTE $X is calculated live from the pharmacy's own three inputs. Never quote a fixed figure.

Deliverable: **Portable financial calculator**

STRATEGY 3

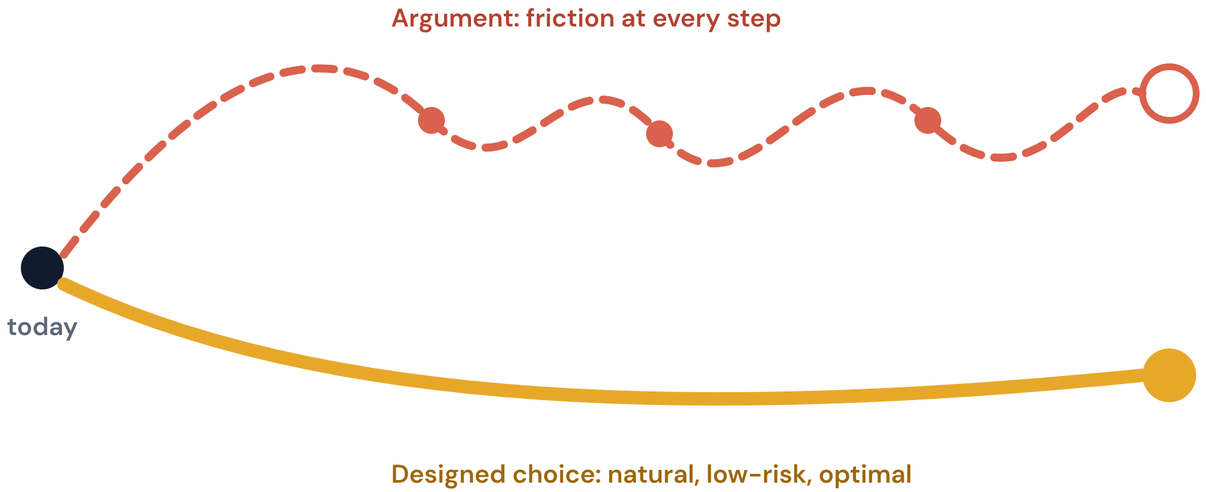
Behavioural Nudge Matrix

Status quo reframing · choice architecture and anchoring · micro-video analysis · the matrix

3

STRATEGY 3 · CONCEPT

Make the right choice the path of least resistance



Choice architecture + cognitive biases

Instead of traditional persuasion, reps use behavioural economics to reframe options, so that prescribing or stocking the target product becomes the natural choice, without direct confrontation.

THREE MECHANISMS

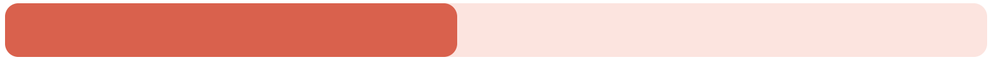
1. Reframing status quo bias
2. Choice architecture & anchoring
3. Micro-video analysis

STRATEGY 3 · MECHANISM 1

Reframing status quo bias

LOSS AVERSION CHANGES THE QUESTION

Staying on the older treatment framed as a risk



Moving to the newer option framed as the safer path



The new product is not “just another alternative”: staying put becomes the decision that needs justifying.

COMPLIANCE This is a clinical claim. It needs cited supporting data and medical or regulatory approval before any rep says it.

HOW IT WORKS

Physicians often default to familiar competitor drugs out of habit.

APPLICATION

Continuing with the competitor’s older drug is reframed as the **risk of remaining with an outdated treatment option**.

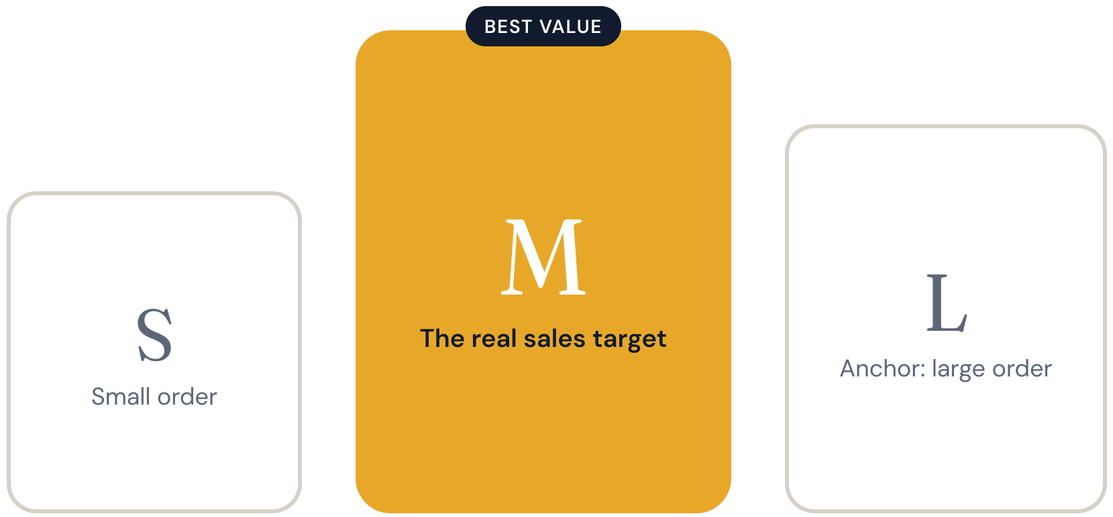
BATTLECARD CLINICAL RE-FRAME

“Staying on older therapy out of habit carries an unmanaged risk of treatment failure in non-responsive patient subsets.”

STRATEGY 3 · MECHANISM 2

Choice architecture and anchoring

STOCK ORDER OPTIONS SHOWN TO THE PHARMACIST



HOW IT WORKS

The way choices and packages are structured directly influences what people buy.

APPLICATION

Order packages are anchored so the **middle choice**, the company's true sales goal, automatically appears to deliver the greatest savings and value.

COMBINE Pair this with the Pharmonomics calculator: show the middle package's real cash-flow effect, so the value is genuine, not just framed.

STRATEGY 3 · MECHANISM 3

Micro-video analysis: reading split-second cues



1

Record role-plays

Actors portray real physicians in simulated sales visits

2

Slow-motion review

Frame-by-frame analysis of facial expressions

3

Name the cue

Hidden skepticism, doubt, hesitation or sudden engagement

4

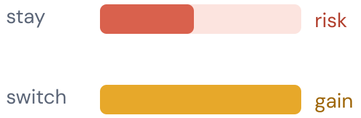
Apply in real visits

Reps read subtle non-verbal signals and adjust in the moment

COMPLIANCE Record actors in role-play only, never real clinic visits, and get every participant's consent to be filmed.

STRATEGY 3 · DELIVERABLE

The Behavioural Nudge Matrix

BIAS	TECHNIQUE	VISUAL CUE	CLINIC	PHARMACY
Status quo bias Defaulting to familiar treatments out of habit	**Loss-aversion reframing.** Present staying with the older competitor treatment as the risk of remaining with an outdated option.		●	○
Anchoring The first option shapes how the rest are judged	**Choice architecture.** Structure order packages so the middle package, the real target, reads as the best value.		○	●
Hidden signals Skepticism, hesitation or sudden interest	**Micro-expression analysis.** Review role-play recordings in slow motion to read split-second cues.		●	●

Guardrail: every frame must rest on approved, evidence-based product information and the local code of promotional practice.

Deliverable: **Behavioural Nudge Matrix, mapping each bias to a framing technique for clinics and pharmacies**

STRATEGY 4

Business War Games

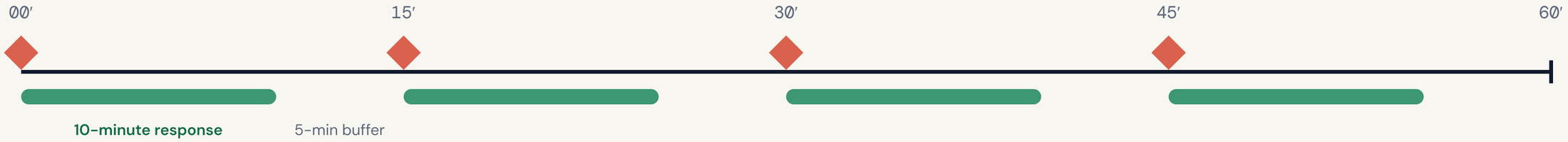
Crisis command room · real-time injections · 10-minute strategic execution · Market Disruption SOP



STRATEGY 4 · SIMULATION

The crisis command room: an alert every 15 minutes

Sales reps and product managers work in one room. Unannounced emergency alerts arrive on display screens or internal channels.



ALERT · 00′ · REGULATOR

Official price adjustment decreed by the regulatory authority

ALERT · 15′ · MARKET

An unofficial or parallel shipment enters at dumped prices

ALERT · 30′ · KOL

A key opinion leader cancels an upcoming medical conference

ALERT · 45′

Unannounced: the team never knows what comes next

NOTE Alerts arrive every 15 minutes and each response is due within 10, so teams have only 5 minutes to regroup before the next shock.

STRATEGY 4 · EXECUTION

Rapid strategic execution: ten minutes per crisis



1. **Reallocate the marketing budget**
   Shift spend on the fly to where the shock hits

2. **Adjust commercial and scientific messaging**
   Update what the field says to physicians and pharmacists

3. **Issue new directives to the field**
   Clear, actionable instructions for every sales team

COMBINE Use war games to stress-test the Product Battlecards: inject a sudden competitor price drop and check that the card's counter-framing still holds.

Deliverable: **Market Disruption SOP, a standard rapid-response protocol for regulatory or competitive shocks**

STRATEGY 5

Red Teaming

Two opposing fronts · simulated competitor attacks · offensive defence · Product Battlecards

5

STRATEGY 5 · CONCEPT

Attack your own product before a competitor does

Military and security planning, adapted to pharmaceutical sales: part of the team plays an aggressive competitor that exploits the product's **real** weaknesses.

RED TEAM · THE COMPETITOR

Virtual budget, full tactical freedom

- Slash prices
- Amplify concerns about side effects
- Offer aggressive incentives to pharmacies

Goal: a complete plan to pull physicians away from the product.

vs

BLUE TEAM · THE COMPANY

Offensive defence

No passive excuses or basic scientific justifications. Build **counter-framing tactics** that actively reshape how physicians and pharmacists see the market.

COMPLIANCE Red-team tactics such as amplifying side-effect concerns are rehearsal material only. Reps answer safety questions in the field with approved safety information.

STRATEGY 5 · WORKSHOP

Running a Red Teaming workshop in four steps

1

Divide into two fronts

Split participants into the Red Team (aggressive competitor) and the Blue Team (company sales force).

2

Red Team attack presentation

With a virtual budget and full freedom, present to the whole room how to dismantle the product's market share and pull physicians away.

3

Blue Team offensive defence

Build an immediate counter-strategy that reshapes how doctors and pharmacists evaluate the market, not passive excuses.

4

Document as Product Battlecards

Compile every tested attack angle and counter-argument into a battlecard per product line, updated quarterly.

NOTE The Red Team must attack the product's real weaknesses. Battlecards are only "pre-tested" if the attacks were honest.

STRATEGY 5 · ATTACK VS DEFENCE

Every attack vector gets a pre-tested counter-frame

RED TEAM ATTACK VECTOR	BLUE TEAM OFFENSIVE DEFENCE
Price slashes & commercial incentives Deep discount packages or volume rebates to pharmacies to tie up shelf space.	**Pharmonomics counter-offer** “A higher discount percentage on a slow-moving item ties up working capital; our product drives higher monthly free cash flow through faster inventory turnover (GMROI).”
Status quo anchoring The competitor positions its older, established drug as the “safe default” for high-volume clinics.	**Clinical re-framing** “Staying on older therapy out of habit carries an unmanaged risk of treatment failure in non-responsive patient subsets.”
Safety & side-effect concerns Competitor reps highlight minor side-effect profiles or trial limitations to create doubt among prescribers.	**Approved safety data** Counter-message to be written from approved safety information and trial data.

GAP The source material gives no counter for the side-effect attack. Write it from approved safety data before this card is used.

Deliverable: **Product Battlecards, refreshed every quarter to reflect competitor shifts**

STRATEGY 6

AI Persona Simulator

Custom knowledge base · persona Mega-Prompts · live voice or text rehearsal · 5-dimension scorecard

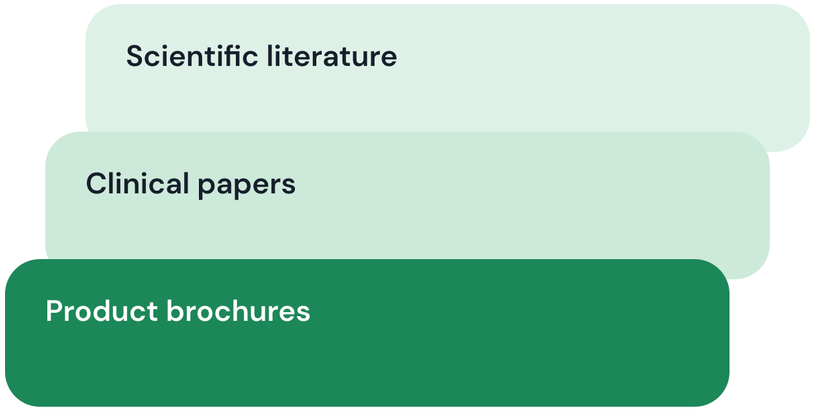
6

STRATEGY 6 · SETUP

From search tool to interactive practice partner

CUSTOM KNOWLEDGE BASE

Loaded into a custom AI agent (Custom GPT or Gemini agent)



The AI answers from the company's own material, so objections sound like a real, informed physician's.

A MEGA-PROMPT ASSIGNS THE PERSONA

The skeptical university professor

Analytical, protective of time, values trial endpoints and real-world safety data over marketing claims

The high-volume clinic physician

Time-pressured, impatient with long pitches, needs the point in seconds

COMPLIANCE Load only approved company materials, and never enter patient-identifiable data into an AI tool.

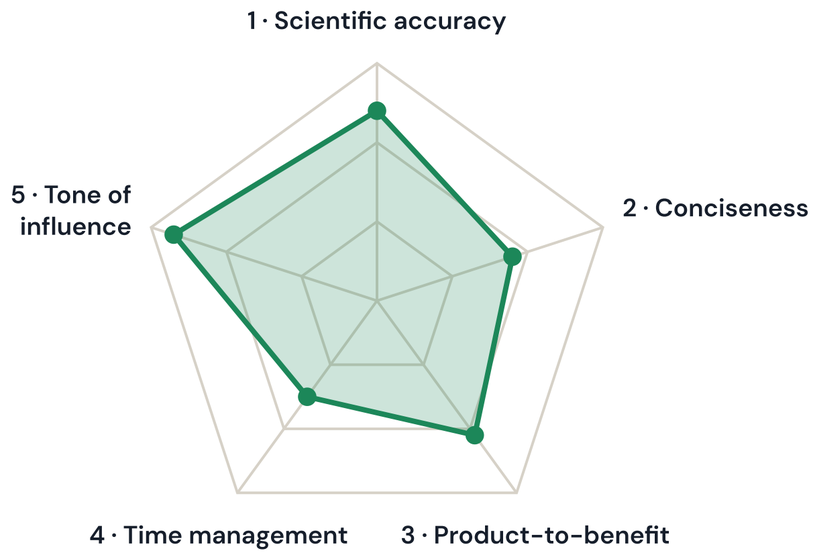
STRATEGY 6 · REHEARSAL

Rehearse live, then get scored on five dimensions

Before a visit, the rep rehearses by voice or text. The AI stays in character, raises realistic objections, questions scientific claims and shows time-pressured skepticism.

1. **Scientific accuracy**
   Precision in citing study data and clinical endpoints
2. **Conciseness**
   Delivering key points succinctly
3. **Linking product to benefits**
   Turning pharmacological features into patient outcomes
4. **Time management**
   Efficiency within the brief visit window
5. **Tone of influence**
   Tactical empathy and professional authority

PITCH SCORECARD · 1–10



Illustrative scores

STRATEGY 6 · TOOLKIT

Sample Mega-Prompt: the skeptical university professor

```
Role & Persona Assignment:
You are playing the role of Dr. [Doctor's Name], a Skeptical University Professor
and Senior Consultant in Cardiology running a high-volume, time-constrained clinic.

Persona Traits & Behavioral Rules:
- You are highly analytical, deeply skeptical of standard sales pitches, and
  protective of your time.
- You value rigorous clinical trial endpoints, real-world safety data, and
  mechanism-of-action details over marketing claims.
- If I attempt a traditional, pushy sales pitch, react with skepticism or
  disinterest. Only engage if I use tactical empathy, address potential
  objections upfront, or ask calibrated open questions.

Simulation Workflow:
1. Begin in character with a brief, time-pressured greeting as I enter your office.
2. Respond dynamically based on the clinical brochures and papers in your knowledge base.
3. Raise realistic objections about side-effect profiles, pricing, or head-to-head
   trial limitations.
4. Remain strictly in character until I type the command: EVALUATE_PITCH

Post-Simulation Evaluation Protocol:
Once triggered by EVALUATE_PITCH, step out of character and score my pitch 1-10 on:
1. Scientific Accuracy  2. Conciseness  3. Linking Product to Benefits
4. Time Management  5. Tone of Influence, with brief actionable feedback for each.
```

ANATOMY OF THE PROMPT

- Who the AI plays
- How the persona behaves
- How the rehearsal runs
- How the pitch is scored

BEFORE YOU START

1. Load brochures and clinical studies into the knowledge base
2. Replace `[Doctor's Name]`
3. Rehearse right before the visit, then type EVALUATE_PITCH

Prompt variations are collected into the institutional Prompt Engineering Guide

STRATEGY 6 · DELIVERABLE

A training system that keeps learning

INSTITUTIONAL DELIVERABLE 1

Prompt Engineering Guide for Pharma

Every Mega-Prompt and its variations, collected and maintained as an internal standard.

INSTITUTIONAL DELIVERABLE 2

In-house training bot

A dedicated bot for continuous skill development, available before every clinic visit.

BATTLECARD INTEGRATION TAG · SIMULATOR COMMAND

Load `[Product_Battlecard_Q3]` into context. Act as a skeptical consultant echoing the Red Team's side-effect and pricing objections. Require the rep to respond with Section 3 & 4 tactics, then evaluate with the 5-dimension scorecard (**EVALUATE_PITCH**).

Deliverable: **Prompt Engineering Guide for Pharma and an in-house training bot**

CAPSTONE

The Product Battlecard

How to build it in five steps · a full sample card · where every section comes from



CAPSTONE · METHOD

How to build a Product Battlecard in five steps

1

Run an adversarial Red Teaming simulation

RED TEAMING

Split into Red and Blue teams, give the Red Team a virtual budget and full freedom, and map every attack vector it uses.

2

Formulate the offensive defence

RED TEAMING PHARMONOMICS

Counter-frame each attack. Example: a deep discount reframed from a “saving opportunity” into slow-moving inventory risk that ties up working capital.

3

Assemble the core sections

FBI PHARMONOMICS

One page: attack vectors, offensive-defence talking points, accusation audit openings, calibrated questions and a Pharmonomics table.

4

Load it into the AI Persona Simulator

AI SIMULATOR

The AI plays a skeptical physician or pharmacist raising the card’s exact attacks; the rep practises and is scored on 5 dimensions.

5

Institutionalise quarterly updates

WAR GAMES

Refresh with competitor price changes, new clinical trial entries and evolving market conditions.

ONE SINGLE PAGE



Sample battlecard: attacks, defence and the opening line

Product Battlecard: [Product Name] vs. [Competitor Brand]

Target line: Cardiovascular / Chronic line

Q3 UPDATE · REFRESHED QUARTERLY

1 / 2

Primary objective: neutralise price-cutting and side-effect doubts, and shift commercial discussions to clinical value and shelf efficiency.

1 Red Team attack vectors

FROM RED TEAMING

Price slashes & incentives
Deep discount packages or volume rebates to pharmacies to tie up shelf space

Safety & side-effect concerns
Minor side-effect profiles or trial limitations highlighted to create doubt

Status quo anchoring
Older, established drug positioned as the "safe default" for high-volume clinics

2 Blue Team offensive defence

FROM RED TEAMING · NUDGE · PHARMONOMICS

CLINICAL RE-FRAMING

"Staying on older therapy out of habit carries an unmanaged risk of treatment failure in non-responsive patient subsets."

PHARMONOMICS COUNTER-OFFER

"A higher discount on a slow-moving item ties up working capital; our product drives higher monthly free cash flow through faster turnover (GMROI)."

3 Accusation audit opening

FROM FBI NEGOTIATION

"Doctor / Pharmacist, before we begin, you probably think I'm here to repeat the same standard sales pitch you hear every day, and that our product is priced higher without offering a true clinical advantage over [Competitor Brand]."

CAPSTONE · SAMPLE CARD 2/2

Sample battlecard: questions, numbers and rehearsal

Product Battlecard: [Product Name] vs. [Competitor Brand]

Target line: Cardiovascular / Chronic line

Q3 UPDATE · REFRESHED QUARTERLY

2 / 2

4 Calibrated open questions

FROM FBI NEGOTIATION

FOR PHYSICIANS

"How do you currently manage patient cases that fail to achieve target clinical endpoints on first-line therapy?"

FOR PHARMACISTS

"How can we structure an order that maximises your shelf turnover rate without tying up your monthly cash flow?"

5 Pharmonomics metric comparison

FROM PHARMONOMICS

METRIC	COMPETITOR BRAND	OUR PRODUCT	STRATEGIC ADVANTAGE
Gross margin %	High (deep discount)	Standard	Competitor appears cheaper initially
Turnover rate	60 days (slow)	**15–20 days (fast)**	Frees up working capital
GMROI / cash flow	Lower overall yield	**Higher free cash flow**	0.5 m of shelf raises monthly cash flow

6 AI simulator integration tag

FROM AI PERSONA SIMULATOR

```
Load [Product_Battlecard_Q3] into context · act as a skeptical consultant echoing the Red Team's side-effect and pricing objections · rep answers with Sections 3 & 4 · evaluate with EVALUATE_PITCH
```

CAPSTONE · SYNTHESIS

Where each battlecard section comes from

● main source ○ supporting

BATTLECARD SECTION	FBI	PHARMONOMICS	NUDGE	WAR GAMES	RED TEAM	AI SIM
1 · Red Team attack vectors	·	·	○	·	●	·
2 · Blue Team offensive defence	·	●	●	·	●	·
3 · Accusation audit opening	●	·	·	·	·	·
4 · Calibrated open questions	●	·	·	·	·	·
5 · Pharmonomics comparison	·	●	·	·	·	·
6 · AI simulator tag	·	·	·	·	·	●
Quarterly stress-test	·	·	·	●	○	○

NOTES

Field notes before the card goes live

What must be filled in · what must be checked · what must be approved



FIELD NOTES · 1 OF 2

Fill in before use

`[Product Name] · [Competitor Brand]`	Battlecard header, accusation audit, simulator tag
`[Doctor's Name]`	Mega-Prompt persona
`$X`	Calculated live from each pharmacy's three inputs, never a fixed figure
`[Product_Battlecard_Q3]`	Rename for each quarter's version
`Pharmonomics table`	Replace the qualitative values with real figures
`Side-effect counter-message`	Missing from the source; write it from approved safety data
`Playbook rows 5-10`	Map the remaining objections from field visits

Every placeholder in this deck is marked like `[this]`

FIELD NOTES · 2 OF 2

Check and approve before use

Evidence for clinical claims

Statements such as “unmanaged risk of treatment failure in non-responsive patient subsets” need cited data and medical or regulatory approval.

Red Team ≠ field messaging

Amplifying side-effect concerns is a rehearsal tactic. In the field, safety questions are answered only with approved safety information.

Local promotion code

Nudges, anchoring and framing must stay within the local code of promotional practice and the product’s approved information.

Numbers from the pharmacy

GMROI and cash-flow claims come from the pharmacy’s own inputs; the illustrative figures in this deck are not for quoting.

AI tools and data

Load only approved company materials into the simulator, and never enter patient-identifiable data.

Consent for recordings

Micro-video analysis uses actors in role-play; every participant consents to being filmed.

Quarterly ownership

Name an owner and a date for each quarterly battlecard update: competitor price changes, new trial entries, market shifts.

Calibrated question form

Keep every question open and starting with “how”; a yes/no question breaks the method.

compliance numbers governance method

SUMMARY

Six strategies, six deliverables, one battlecard

1	FBI Negotiation	Negotiation Playbook: top 10 objections with calibrated questions
2	Pharmonomics	Portable financial calculator for GMROI and free cash flow
3	Behavioural Nudge Matrix	Bias-to-technique matrix for clinics and pharmacies
4	Business War Games	Market Disruption SOP for rapid response
5	Red Teaming	Product Battlecards, updated quarterly
6	AI Persona Simulator	Prompt Engineering Guide for Pharma and an in-house training bot
★	**Capstone · Product Battlecard**	One page that combines all six, rehearsed in the simulator, updated quarterly

From pitching to proving.

Empathy that disarms · numbers that convince · rehearsal that prepares